

Our Ref: BGM/bp (T) // 0096218

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

INTERSTITIAL LUNG DISEASE

[PROVIDER-NAME] Consultant Respiratory Physician

[PROVIDER-NAME] Consultant Respiratory Physician and Honorary Associate Professor in Medicine

[PROVIDER-NAME] Consultant Respiratory Physician

[PROVIDER-NAME] Consultant Respiratory Physician

Division B

Care Group - Emergency & Specialist Medicine Respiratory Medicine

CG89 Centre Block, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

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Dear [PROVIDER-NAME] [NAME] , [ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses:

Stage III pulmonary sarcoidosis (histological diagnosis of mediastinal lymph node 2015)

Crohn's disease (jejunal stricture resection 2015)

I saw Mr [NAME] today who looks well but still feels that his symptoms particularly fatigue, and lack of energy continue. He gets occasional sweats and also feels nauseous.

These seem to have got worse his prednisolone which was reduced six months ago.

He has gained 10 kilograms unfortunately since diagnosis on the prednisolone-azathioprine regime currently on prednisolone 10 milligrams once a day and azathioprine 25 milligrams once a day.

He does not report significant GI problems at the moment. A chest x-ray and lung function tests shows stable appearances with no progression and his recent bloods at the end of last month show inflammatory response and normal biochemistry profile and full blood count.

I have asked him if he could increase his azathioprine dose to 50 milligrams once a day for three months and we shall see him at that stage to assess for subjective improvement. I will see him in four months time with repeat blood tests.

Yours sincerely

[NAME]

[DICTATION-META]

[PROVIDER-NAME]

Consultant Respiratory Physician and

Honorary Associate Professor in Medicine

Copy to:

[ADDRESS]

E Level

West Wing

SGH